

RADIOLOGY REPORT

Date: 2026-04-23 06:06 UTC

PATIENT INFORMATION

Name: Test Patient
Date of Birth: 1995-04-23
Gender: Male
Phone: +2010203040

IMAGING DETAILS

Modality: XRay
Priority: Low
Emergency: No

CLINICAL HISTORY

45-year-old male presenting with persistent lower back pain radiating to the left leg for 3 weeks. No history of trauma. Previous lumbar X-ray was unremarkable.

TECHNIQUE

MRI of the lumbar spine was performed using T1-weighted, T2-weighted, and STIR sequences in sagittal and axial planes without contrast administration.

FINDINGS

L4-L5 disc shows posterior central disc herniation measuring approximately 6mm, causing mild compression of the thecal sac. There is mild bilateral foraminal narrowing at L4-L5. The conus medullaris terminates at the L1 level and appears normal. No evidence of spinal cord signal abnormality. Remaining intervertebral discs show normal signal intensity and height. Vertebral body heights and alignment are maintained. Paraspinal soft tissues appear unremarkable.

IMPRESSION

- Posterior central disc herniation at L4-L5 with mild thecal sac compression.
- Mild bilateral foraminal narrowing at L4-L5.
- No evidence of spinal cord compression or signal abnormality.

RECOMMENDATION

Clinical correlation with neurology consultation is recommended. Conservative management with physical therapy is suggested as initial treatment. Follow-up MRI in 6-8 weeks if symptoms persist or worsen. Surgical evaluation may be considered if conservative management fails.